

How to Open an Outpatient Treatment Center in Arizona

A step-by-step guide to licensing and opening a behavioral health OTC — outpatient counseling, IOP, partial hospitalization, and medication-assisted treatment — and getting paid by AHCCCS

PolicyReady.org — Practical Playbooks for Care Entrepreneurs

What an OTC is — read this first

An Outpatient Treatment Center (OTC) is a licensed behavioral health clinic with no beds. Patients come in for services — assessment, counseling, medication management, intensive outpatient, partial hospitalization, and medication-assisted treatment — and go home the same day. In Arizona it is a class of health care institution licensed by the Department of Health Services under Article 10, and it is paid by AHCCCS (Arizona Medicaid) largely per service rather than by a daily rate. Because there are no residents, an OTC skips the room-and-board problem that defines a residential facility — but it takes on its own: a real clinical license, licensed clinicians, and per-service billing that has to be documented and coded correctly. Opening one means clearing two gates: the ADHS license and the AHCCCS contract.

How to Use This Guide

This guide takes you from idea to a licensed, AHCCCS-contracted outpatient behavioral health clinic. Read it once start to finish, then use it as a checklist. Everything here is general information for prospective Arizona operators — not legal, clinical, or billing advice. OTC rules are detailed and are actively updated by ADHS and AHCCCS, so verify each step with the agency named before you rely on it. Where this guide gives a code, a fee, or a figure, treat it as a starting point to confirm.

The one-paragraph version

Form an Arizona business entity and secure a clinic space that can pass the physical-plant and fire requirements for an outpatient health care institution. Apply to ADHS for an Outpatient Treatment Center license under Article 10 — filed electronically through the state's licensing system — and decide which behavioral health services you will offer, from standard outpatient counseling up through intensive outpatient, partial hospitalization, and medication-assisted treatment. Build your clinical team: an administrator, licensed behavioral health professionals credentialed by the Arizona Board of Behavioral Health Examiners, a medical director or collaborating physician if you provide medication or MAT, and support staff, all with Fingerprint Clearance Cards. Enroll with AHCCCS and contract with the AHCCCS Complete Care health plans. Understand how you get paid: outpatient behavioral health is billed largely per service on a professional claim, with per-diem rates for intensive outpatient and partial hospitalization. Protect your records under HIPAA and, for substance use, the stricter federal 42 CFR Part 2 rule. Then run the clinic to the documentation and quality standards AHCCCS and ADHS require.

The Path at a Glance

Ten steps take you from idea to an operating Arizona OTC:

1. **Decide your services.** Which behavioral health services — outpatient counseling, IOP, PHP, medication management, MAT — and for which population.

2. **Form the business.** Register your entity with the Arizona Corporation Commission and get an EIN.
3. **Secure the space.** A clinic that can meet the outpatient physical-plant and fire-safety requirements — no beds needed.
4. **Write your policies.** Adopt the OTC policy and procedure manual, clinical forms, and the documentation the license requires.
5. **Build your team.** Administrator, AZBBHE-licensed behavioral health professionals, a medical director for medication/MAT, and support staff — all fingerprint-cleared.
6. **Apply to ADHS.** Submit the initial Behavioral Health Facility License application for an OTC under Article 10 through the electronic licensing system, with fees and drawings.
7. **Pass the inspection.** ADHS reviews your application and inspects the clinic before issuing the license.
8. **Enroll with AHCCCS.** Register as an AHCCCS provider through APEP and contract with the AHCCCS Complete Care health plans.
9. **Set up billing & records.** Stand up your clinical documentation, coding, HIPAA, and 42 CFR Part 2 protections for substance use records.
10. **Open and see patients.** Assess, treat, document, and bill — building referral relationships across the behavioral health system.

Part 1 — What This Business Is

An Outpatient Treatment Center is a behavioral health clinic — a place people come to for treatment of a mental health or substance use condition and then return home. There are no beds and no overnight stays. What makes an OTC valuable is that it can offer a whole ladder of outpatient services under one license, from a weekly counseling appointment all the way up to structured day programs that are the last step before, or the first step after, residential or hospital care.

The service ladder

- **Standard outpatient.** Assessment and diagnosis, individual, group, and family counseling, and medication management — the core of most clinics.
- **Intensive Outpatient (IOP).** A structured program of several hours, several days a week — more support than weekly therapy, without living at a facility.
- **Partial Hospitalization (PHP).** The most intensive outpatient level — a near-daily day program for people who need a lot of structure but can safely go home at night.
- **Medication-Assisted Treatment (MAT).** Medications for opioid or alcohol use disorder combined with counseling; office-based buprenorphine and naltrexone fit an OTC, while methadone requires a separate opioid-treatment-program setting.
- **Telehealth.** Arizona supports delivering much of outpatient behavioral health by video, which extends your reach — especially into rural areas.

Arizona's demand for outpatient behavioral health is deep and growing, and AHCCCS funds the full ladder. An OTC that offers more than one rung — say, standard outpatient plus IOP plus MAT — can meet patients where they are and keep them in your care as their needs rise and fall, which is both better clinically and steadier as a business.

Why Arizona, and why an OTC

Two things make Arizona attractive for outpatient behavioral health. First, the need: the state has significant behavioral health and substance use demand, an opioid crisis it has invested heavily in addressing, and large rural areas that are chronically short of providers — which is exactly where telehealth-capable outpatient clinics can reach. Second, the funding: AHCCCS covers the full continuum of outpatient behavioral health, so a licensed, contracted OTC has a real, ongoing payer rather than depending only on private pay. The result is a market where a well-run clinic that keeps its schedule full and its documentation clean can build a durable practice.

Compared with a residential facility, an OTC is lighter to open in some ways and heavier in others. Lighter: no beds, no 24-hour staffing, no room-and-board problem, and a smaller physical build-out. Heavier: your revenue is earned one service at a time, so your billing and documentation have to be excellent from day one, and you live or die on having enough credentialed clinicians with full schedules. Go in understanding that an OTC is a professional practice — closer to running a busy clinic than owning a building.

The economics, roughly

An OTC's economics are a volume-and-mix business. You employ or contract clinicians, you bill for the services they deliver, and the margin between what you collect per service and what you pay your clinicians and overhead is your profit. Because most services are billed per encounter, two levers matter most: keeping clinicians' schedules full (empty slots earn nothing) and getting clean claims paid the first time (denials and rework quietly destroy margin). Adding higher-intensity rungs like IOP and PHP, which bill as per-diem programs, can raise revenue per patient-day; adding MAT meets a major need and brings medication and prescriber visits into the mix. Model conservatively — assume ramp-up time to fill schedules, assume some no-shows, and make sure your rates and volume cover clinician pay, rent, billing, and compliance with room to spare.

What an OTC is not

- **Not a BHRF.** A Behavioral Health Residential Facility (Article 7) is 24-hour residential treatment with beds. An OTC has no beds and no room-and-board question.

- **Not an inpatient facility.** A behavioral health inpatient facility is hospital-level care. An OTC is strictly outpatient.
- **Not just a counseling facility.** Arizona also licenses a lighter Counseling Facility class (Article 19). An OTC is the fuller outpatient class, and an OTC can provide administrative support to counseling facilities under the same ownership — a common way to grow.

Part 2 — The Two Gates: the ADHS License and the AHCCCS Contract

As with any Arizona behavioral health facility, opening an OTC comes down to two approvals, and you need both. The first is the license from the Arizona Department of Health Services, which lets you legally operate an outpatient behavioral health clinic. The second is the provider agreement with AHCCCS and its health plans, which lets you get paid — because most of your patients will be AHCCCS members whose care is funded through the behavioral health system.

Treat these as two parallel tracks, not one. A license without an AHCCCS contract is a clinic with clinicians and no way to bill Medicaid; an AHCCCS enrollment without a license is paperwork with no clinic behind it. Run both at once, and expect the whole path — space, license, and contracts — to take months.

The mindset that keeps an OTC open

An OTC lives on documentation and coding. Unlike a residential per-diem, outpatient revenue is earned service by service — each assessment, session, and program day is a billable event that has to be medically necessary, documented, and coded correctly. The clinics that thrive build clean clinical documentation and billing into their daily rhythm from day one, because that is literally how they get paid, and it is what an AHCCCS audit checks.

Part 3 — Getting Licensed by ADHS (Article 10)

The Arizona Department of Health Services licenses outpatient treatment centers under Arizona Administrative Code, Title 9, Chapter 10, Article 10. An OTC that only provides behavioral health services is licensed as a behavioral health facility. You apply on the behavioral health facility license application, and ADHS reviews your application and inspects your clinic before issuing a license.

What the application involves

- **Electronic filing.** Arizona no longer accepts paper applications — the initial application, renewals, and amendments are filed electronically through the state's licensing management system. Build your document package as uploadable files from the start.
- **The application and fees.** Expect a non-refundable application fee (recently \$50) plus the license fees set by rule, and, if you build or renovate, an architectural plan-review fee scaled to project cost. Fees are non-refundable, so submit a complete, correct packet. File it within the window before your planned opening.
- **The clinic space.** The space must meet the outpatient physical-plant standards for an OTC — treatment and counseling rooms, privacy, and safety. Because there are no beds, this is lighter than a residential build-out, but it is real. If you construct or modify space, submit architect-signed drawings that meet the adopted building and fire codes.
- **Fire and occupancy.** You need a passing fire and life-safety inspection and a current certificate of occupancy for the use.
- **Ownership and controlling persons.** ADHS reviews the applicant and controlling persons; a disqualifying history can bar a license.

After a complete application, ADHS runs its completeness and substantive reviews within the timeframes in its rules, inspects the clinic, and then approves or denies. A realistic plan is several months — often three to six — from a complete submission to an issued license, depending on your documentation and whether the inspection surfaces anything to fix. Do not see patients or bill before the license is issued.

Two practical points smooth the licensing track. First, submit a genuinely complete electronic packet the first time — the usual causes of delay are a missing fire inspection or certificate of occupancy, incomplete policies, or unresolved physical-plant items, and because the fees are non-refundable, a rejected packet costs both time and money. Have your policy and procedure manual, clinical documentation forms, staffing plan, and clinician credentials assembled before you file, because ADHS wants evidence the clinic will actually operate to the rules. Second, remember the license names your class, your services, your location, and your controlling persons — adding a service line or a site later is its own amendment, so scope your initial application to what you can open with and plan expansions deliberately.

OTC, counseling facility, or affiliated OTC?

Decide your structure before you apply. A single OTC is the straightforward path. If you plan to run multiple counseling sites, you can license an OTC that provides administrative support to counseling facilities (Article 19) under the same ownership — an affiliated OTC — which is how many groups scale. Choosing the right structure up front saves you from re-licensing later.

Part 4 — Building Your Clinical Team

An OTC is a clinical operation, and Arizona expects it to be staffed and supervised by qualified people. Line these roles up before you apply, because ADHS wants to see that the clinic will be properly run.

- **Governing authority and administrator.** The governing authority is legally responsible for the clinic; the administrator runs day-to-day operations and meets the qualifications in the rules.
- **Behavioral Health Professionals (BHPs).** Your licensed clinicians — licensed professional counselors, licensed clinical social workers, licensed marriage and family therapists, licensed independent substance abuse

counselors, psychologists, or psychiatrists — credentialed by the Arizona Board of Behavioral Health Examiners (or the relevant board). They assess, diagnose, treat, and supervise.

- **Medical director or collaborating physician.** If you provide medication management or medication-assisted treatment, you need a physician (often a psychiatrist) or an appropriately licensed prescriber, with the required authority for controlled substances used in MAT.
- **Behavioral health technicians and paraprofessionals.** Support staff who deliver services under professional supervision, trained and qualified as the rules require.
- **Fingerprint Clearance Cards.** Staff must hold Arizona Fingerprint Clearance Cards and clear background screening before they work with patients.

Credentialing is a second clock

Your clinicians do not just need to be hired — they need to be credentialed with each AHCCCS health plan before their services bill cleanly, and that takes time. Start clinician credentialing in parallel with your license and your plan contracts, or you will have a licensed clinic full of clinicians whose claims are not yet payable. Plan the credentialing calendar as deliberately as the license.

Part 5 — How Care Works: Assessment to Discharge

Outpatient care follows a clear arc, and because you bill per service, the clinical arc and the billing arc are the same thing. Understanding it keeps both your care and your revenue clean.

- **Intake and assessment.** A behavioral health assessment establishes the diagnosis and medical necessity that justify services. This is the foundation — weak intake documentation undermines everything billed after it.
- **Service plan.** A treatment plan sets goals and the services to reach them, at the right level of the ladder for the patient's needs.
- **Delivering services.** Counseling, group work, medication management, and — where offered — IOP, PHP, or MAT, each documented as delivered and reviewed as the patient progresses.
- **Level-of-care changes.** A patient may step up (to IOP or PHP) or step down (to standard outpatient) as they improve or struggle; an OTC that offers several rungs can keep them in care through those changes.
- **Coordination and discharge.** Coordinate with the patient's health plan, primary care, and any referring provider, and discharge or transition when goals are met or a different level of care is needed.

MAT belongs in the outpatient world

Medication-assisted treatment for opioid and alcohol use disorder is a natural fit for an OTC and a major access need in Arizona. Office-based buprenorphine and naltrexone can be offered in an OTC with an appropriate prescriber; methadone for opioid use disorder must be delivered through a licensed opioid treatment program, which is a distinct setting. If MAT is part of your model, build the prescriber relationship, the counseling component, and the controlled-substance compliance in from the start.

Where your patients come from

An OTC fills its schedule through the behavioral health system and the broader community, so knowing who refers — and being easy to refer to — is how you stay busy. Your referral sources include:

- **The AHCCCS Complete Care health plans.** Their care managers steer members to in-network outpatient providers; being contracted, credentialed, and responsive keeps you in the referral flow.
- **Primary care and hospitals.** Primary care practices refer patients who need behavioral health, and hospitals discharge patients to outpatient follow-up to prevent readmission.
- **The crisis system.** People stabilized in crisis need somewhere to continue care; an OTC that accepts referrals quickly becomes a reliable landing spot.
- **Residential and inpatient facilities.** BHRFs and hospitals discharge patients stepping down to outpatient care — a natural, steady referral stream, especially if you offer IOP or PHP.
- **Courts, probation, and schools.** Justice-involved and school-referred clients are a consistent source, particularly for substance use and youth services.

The common thread is responsiveness. Referral sources send patients to clinics that answer the phone, schedule quickly, communicate back, and follow through. A reputation for being easy to work with, built with a handful of health-plan care managers, hospital discharge planners, and primary care offices, is in practice what keeps your schedule full.

Part 6 — Getting Paid

Most OTC patients are AHCCCS (Arizona Medicaid) members, so AHCCCS is your payer. Getting paid means enrolling, contracting, and billing outpatient behavioral health correctly — which, unlike a residential per-diem, is mostly a per-service system.

Enroll and contract

Register as an AHCCCS provider through the AHCCCS Provider Enrollment Portal (APEP) as the appropriate behavioral health outpatient provider type, then contract with the AHCCCS Complete Care (ACC) health plans whose members you want to serve, and understand the fee-for-service path for members who are not in a plan. Enrolling with AHCCCS is not the same as contracting with a plan or credentialing your clinicians — you generally need all three before claims pay.

The three-part path — enroll, contract, credential — is worth planning as three separate calendars that overlap. Enrollment makes the clinic a recognized AHCCCS provider. Contracting brings each ACC plan's members and rates. Credentialing verifies each individual clinician with each plan so their services are payable. New OTCs routinely underestimate credentialing: a fully licensed clinic with hired clinicians can still be unable to bill for weeks because the clinicians are not yet credentialed with the plans. Start all three early and track them together, and consider a billing partner or clearinghouse that knows Arizona behavioral health if you do not have that expertise in-house.

How outpatient billing works

Outpatient behavioral health is billed on the professional claim (CMS 1500 / 837P), mostly per service: an assessment, a therapy session, a group, a medication visit. Intensive outpatient and partial hospitalization are billed as per-diem programs. Rates are set by AHCCCS and each health plan, and many services carry modifiers that identify who rendered them and how. There is no room-and-board question here — an OTC bills for the services it delivers, not for a bed. As a starting reference:

Service	Common code	Notes
Behavioral health assessment / diagnostic eval	90791 (or 90792 with medical)	Establishes diagnosis & medical necessity
Individual counseling	90832 / 90834 / 90837; H0004	By session length or per 15 minutes
Group counseling	90853; H0005	Group-based; each member billed individually
Family therapy	90846 / 90847	With or without the patient present
Intensive Outpatient (IOP)	H0015 (per diem)	Structured program; per-day rate
Partial Hospitalization (PHP)	H0035 (per diem)	Most intensive outpatient level; per-day rate
Medication management	E/M (99xxx); 90863	Prescriber visits
Office-based MAT / OTP	G2086–G2088; G2067–G2075; H0020	Bundles for opioid treatment; methadone in OTP setting

Codes are national references; AHCCCS and your health plan set the exact code, modifier, rate, and rules. Verify before billing.

Finding your actual rates

This guide does not print rate dollar figures on purpose — outpatient behavioral health has many codes, and the amount you are paid depends on the code, the rendering provider, the modifiers, and, above all, your contract. AHCCCS publishes dated behavioral health fee schedules on its Rates and Billing pages in two versions, fee-for-service (FFS) and managed-care (MCO); you can look up a specific code on the current-dated schedule, but treat the FFS figure as a reference because your contracted ACC plans set the rates they actually pay you. AHCCCS states plainly that a listed rate is not a guarantee of payment. To pin down your numbers, look up the codes on the current AHCCCS behavioral health fee schedule, ask each ACC plan for its rates, and email AHCCCS at FFSRates@azahcccs.gov with rate questions.

The payer mix

Source	What it covers	What to know
AHCCCS (ACC plans / FFS)	The bulk of outpatient behavioral health	Requires enrollment, plan contracts, and clinician credentialing
Commercial insurance	Outpatient services for insured patients	Separate contracting and credentialing with each payer
Private pay / sliding scale	Self-paying patients	Set clear rates; a sliding scale can widen access
Grants / state & county funding	Gaps, uninsured, special programs	Behavioral-health funding streams support specific services and populations

Part 7 — The Space, Records & Privacy

An OTC's physical requirements are lighter than a residential facility's — you need appropriate treatment and counseling rooms, privacy for clinical conversations, accessibility, and a space that passes fire and life-safety inspection and holds a certificate of occupancy for the use. But records and privacy carry extra weight in behavioral health, and getting them wrong is a serious risk.

Substance use records have a stricter rule than HIPAA

Know 42 CFR Part 2. On top of HIPAA, federal law (42 CFR Part 2) gives special, stricter confidentiality protection to substance use disorder treatment records — with tighter consent rules for sharing information. If your OTC treats substance use, build Part 2-compliant consent and records handling into your operations from the start, train your staff on it, and make sure your electronic health record supports it. Mishandling Part 2 records is both a legal exposure and a breach of patient trust.

Also plan your telehealth setup deliberately: Arizona supports delivering much of outpatient behavioral health by video, which can widen access and fill your schedule, but it comes with its own consent, documentation, and technology-security requirements. And while extreme heat is less central for an outpatient clinic than a residence, your emergency plan should still cover the Arizona basics — heat, power outages, and continuity of care if the clinic closes unexpectedly.

Part 8 — Your Startup Sequence

1. **Months 1–2:** Form the entity, get your EIN, and secure a clinic space that can meet outpatient physical-plant and fire requirements. Decide your service ladder.
2. **Months 1–3:** Recruit your administrator, BHPs, and (for medication/MAT) a prescriber; adopt your OTC policy manual and clinical documentation; start Fingerprint Clearance Cards.
3. **Months 2–4:** Complete any build-out with architect-signed drawings; pass fire and occupancy; submit the ADHS Article 10 application electronically with fees and drawings.
4. **Months 3–6:** Pass the ADHS inspection and receive your license. In parallel, enroll with AHCCCS through APEP, contract with the ACC plans, and credential your clinicians.
5. **Months 5–6:** Stand up billing, HIPAA, and 42 CFR Part 2 protections; build referral relationships; and open your doors to patients.

Part 9 — Common Mistakes to Avoid

- **Treating the license as the finish line.** Without the AHCCCS contract and credentialed clinicians, a licensed clinic still cannot bill Medicaid.
- **Forgetting clinician credentialing.** Being licensed and hired is not enough; each clinician must be credentialed with each plan before claims pay.
- **Weak intake documentation.** If the assessment and medical necessity are thin, everything billed afterward is exposed on audit.
- **Coding and modifier errors.** Outpatient behavioral health is modifier-heavy; the wrong rendering provider or program code is a preventable denial.
- **Mishandling 42 CFR Part 2 records.** Substance use records need stricter consent than HIPAA; getting it wrong is a real legal exposure.
- **Confusing an OTC with a counseling facility.** They are different classes; choose the structure that fits your plans before you apply.
- **Offering MAT without the compliance.** Medication-assisted treatment needs the right prescriber, counseling, and controlled-substance compliance — not just a good intention.

Part 10 — Staying Licensed & Compliant

Once you are open, compliance is continuous. Keep your ADHS license current and pass relicensure inspections; keep every clinician's board license, your staff's Fingerprint Clearance Cards, and your credentialing current; maintain complete, timely clinical documentation that supports every service billed; protect records under HIPAA and 42 CFR Part 2; keep authorizations current where services require them; report suspected abuse, neglect, and exploitation as the rules require (to Arizona Adult Protective Services at 1-877-767-2385 for adults, and the Department of Child Safety for minors); and keep your AHCCCS enrollment and plan contracts in good standing. Run a quality program that tracks outcomes, access, documentation, denials, and audit findings, and fix what it shows.

The bottom line

An OTC is a clinical business that earns its revenue service by service. Clear the two gates — the ADHS Article 10 license and the AHCCCS contract — staff it with credentialed clinicians, offer a ladder of services you can bill and document cleanly, protect your records, and coordinate across the behavioral health system. Do those, and you have a clinic that meets a deep need and stays busy. Skip the documentation, coding, or credentialing, and the demand never turns into paid claims.

Two Worked Examples

Example 1 — A counseling-focused OTC

You open an OTC offering assessment, individual, group, and family counseling, and medication management for adults with mental health conditions. You hire an administrator and a team of AZBBHE-licensed counselors and social workers, contract with a psychiatric nurse practitioner for medication visits, and fingerprint-clear everyone. You pass the ADHS inspection, enroll with AHCCCS, contract with two ACC plans, and credential your clinicians. Referrals come from primary care, hospitals discharging patients to outpatient follow-up, and the crisis system. You bill assessments, sessions, and medication visits per service on professional claims, document every encounter, and coordinate with each patient's health plan and primary care.

Example 2 — An OTC with IOP and MAT

You build an OTC around substance use treatment, offering standard outpatient plus an intensive outpatient program and office-based medication-assisted treatment. Your prescriber provides buprenorphine and naltrexone alongside counseling; you route methadone needs to a licensed opioid treatment program. You bill the IOP per-diem for program days and per-service codes for counseling and medication visits, and you run your substance use records under 42 CFR Part 2. As patients stabilize, they step down from IOP to standard outpatient and stay in your care. Your referral partners are hospitals, the crisis system, courts and probation, and residential facilities discharging patients to outpatient step-down.

Who to Call & What to Ask

For...	Who to contact
Licensing an OTC	ADHS behavioral health facility licensing, 602-542-1025 (licensing.azdhs.gov). Ask: what does a complete Article 10 OTC application require, and what are the current fees and timeframes?
Clinician credentials	Arizona Board of Behavioral Health Examiners (azbbhe.us). Ask: what licenses do my clinicians need, and how do they verify and renew them?
The space & fire	Your local fire department / fire marshal and building department, and an architect familiar with health-care-institution codes. Ask: what do I need to pass fire and occupancy for an outpatient clinic?
Fingerprint Clearance Cards	Arizona Department of Public Safety (azdps.gov). Ask: how do my staff apply for Level One cards?
Becoming an AHCCCS provider	AHCCCS Provider Enrollment (APEP), azahcccs.gov . Ask: how do I enroll as an outpatient behavioral health provider, and how do I credential my clinicians?
Getting paid	The AHCCCS Complete Care health plans and AHCCCS Provider Services, 602-417-7670. Ask: how do I contract, and what are your outpatient behavioral health billing and authorization rules?
MAT / controlled substances	Your prescriber, the DEA, and the Arizona Board of Pharmacy. Ask: what do I need to offer office-based medication-assisted treatment?
Reporting abuse	Arizona Adult Protective Services — 1-877-767-2385 (adults); Department of Child Safety — 1-888-767-2445 (minors); 911 for emergencies.

Key Terms

- **OTC:** Outpatient Treatment Center — a licensed behavioral health clinic with no inpatient beds.
- **Article 10:** the ADHS licensing rules for outpatient treatment centers (A.A.C. Title 9, Chapter 10, Article 10).
- **Counseling facility / affiliated OTC:** a lighter Article 19 outpatient class, and an OTC that provides administrative support to counseling facilities under the same ownership.
- **BHP:** Behavioral Health Professional — a board-licensed clinician (LPC, LCSW, LMFT, LISAC, psychologist, psychiatrist).
- **AZBBHE:** the Arizona Board of Behavioral Health Examiners, which licenses most behavioral health clinicians.
- **IOP / PHP:** Intensive Outpatient and Partial Hospitalization — the structured, higher rungs of outpatient care, billed per diem.
- **MAT / OBOT:** Medication-Assisted Treatment / Office-Based Opioid Treatment — medication plus counseling for substance use.
- **42 CFR Part 2:** the federal rule giving substance use disorder records stricter confidentiality than HIPAA.
- **APEP:** the AHCCCS Provider Enrollment Portal, where you enroll and revalidate as a provider.
- **ACC plan:** an AHCCCS Complete Care managed-care health plan you contract with to serve its members.

Core Rules & References

- A.A.C. Title 9, Chapter 10, Article 10 — ADHS licensing of Outpatient Treatment Centers (R9-10-1001 et seq.)
- A.A.C. R9-10-101 — definitions, including “outpatient treatment center” and “behavioral health facility”
- A.A.C. Title 9, Chapter 10, Article 19 — Counseling Facilities (the related lighter class)
- A.R.S. § 36-422 & A.A.C. R9-10-106 — health-care-institution license application and fees
- A.A.C. R9-10-1030 — physical plant, environmental services, and equipment standards for an OTC
- AHCCCS AMPM & the Covered Behavioral Health Services Guide — outpatient service coverage and coding
- 42 CFR Part 2 & HIPAA — confidentiality of substance use disorder records and health information
- A.R.S. § 46-454 (adults) & § 13-3620 (minors) — mandatory reporting of abuse, neglect, and exploitation

This guide is a general educational resource for prospective Arizona OTC operators. It is not legal, clinical, tax, or billing advice, and requirements, fees, codes, and contacts change. Verify every step with the agency named — ADHS, the Arizona Board of Behavioral Health Examiners, AHCCCS and the health plans, and the local fire and building departments — before you rely on it. Provisions cited reflect Arizona law and AHCCCS policy at the time of writing.